

Follow-up Fetal Echocardiography

Sonographer + physician one-page cheat sheet

ORDER > ACQUIRE > INTERPRET > DOCUMENT > CODE

All five should tell the same clinical story.

CORE RULE: Follow-up fetal echo is a dedicated reassessment of a previously evaluated fetal cardiovascular condition, not simply another look at the heart during an obstetric ultrasound.

1 QUALIFY THE EXAM

- ☐ **Target:** What lesion, functional issue, rhythm problem, or conduction risk is being reassessed?
- ☐ **Baseline:** Record the prior complete fetal echo date, location, and major findings.
- ☐ **Today's question:** Resolved, stable, improved, progressed, or new finding?

No prior complete fetal echo? Do not assume follow-up. Determine whether an initial complete fetal echo is appropriate.

CODE ONLY SUPPORTED COMPONENTS

76826 Follow-up 2D fetal cardiac reassessment**76828** Stored spectral Doppler waveforms**93325** Color-flow velocity mapping

HARD STOP: No stored spectral waveform = no 76828. Color Doppler is not spectral Doppler.

2 SONOGRAPHER: ACQUIRE

2D / GRAYSCALE - 76826

- ☐ Dynamic cine loops, not only still images
- ☐ 4-chamber, LVOT, RVOT, 3-vessel / 3VT
- ☐ Aortic + ductal arches; venous anatomy as indicated
- ☐ Ventricular size, symmetry, function; lesion views + measurements

COLOR FLOW - 93325

- ☐ Relevant septa, valves, outflows, arches, shunts, and veins
- ☐ Direction, turbulence, acceleration, and regurgitation
- ☐ Optimize box, scale/PRF, gain, and frame rate

SPECTRAL DOPPLER - 76828

- ☐ Store actual waveforms selected for the lesion
- ☐ Inflow, outflow, arches, veins, abnormal jet/vessel as indicated
- ☐ Mechanical PR when indicated; optimize beam alignment

ALWAYS: heart rate/rhythm, chamber proportionality, ventricular and valve function, relevant flow direction, deterioration, and hydrops when appropriate.

3 PHYSICIAN: REPORT

WHY?

Specific cardiac reassessment indication

BASELINE?

Prior complete echo date, location, and findings

PERFORMED?

Only today's actual 2D, M-mode, color, spectral, and measurements

FOUND?

Anatomy, function, hemodynamics, rhythm, measurements

CHANGED?

Resolved, stable, improved, progressed, or new finding

REPORT STARTER: "Follow-up fetal echocardiogram for [condition]. Prior complete study on [date] showed [baseline]. Today, [components] showed [findings]. Compared with prior: [interval change]."

SAME-DAY OB + ECHO: Both may be necessary. Keep indications, acquisition, and reports distinct. Verify current payer edits, modifiers, bundling, and coverage rules.

LESION-SPECIFIC PROMPTS

PERICARDIAL EFFUSION

- Consistent measurement
- Function + rhythm
- AV regurgitation
- Hydrops / other fluid

VSD / SEPTAL DEFECT

- Location + size
- 2D + color flow
- Ventricular proportion
- Outflows / arches

VALVE DISEASE

- Morphology + color
- Spectral velocity
- Regurgitation
- Size/function + trend

CARDIOMYOPATHY

- Wall/septal thickness
- Chamber size
- Systolic function
- Doppler + hydrops

ARRHYTHMIA

- Rate + rhythm
- A-V relationship
- M-mode/Doppler
- PR/function/hydrops

10-SECOND CLOSEOUT

SONOGRAPHER: cine | lesion | color | spectral if indicated | measurements | function | rhythm

PHYSICIAN: indication | prior echo | components | findings | interval change | plan

